

Cardiopulmonary Assessment

Sedative Physio

- **Demographic Data**

- **Name**
- **Age**
- **Sex**
- **Marital status**
- **Address**
- **Occupation**

- **Chief Complaint**

- **Should be mentioned in the patient's language**

History

- **Present History**
 - **Onset**
 - **Duration**
 - **Associated problems**
- **Past medical history**
 - **Any medical condition**
 - **Any trauma/RTA**
 - **Any Pathological condition**
- **Surgical History**

On Observation

- **General apperance**
- **Built of patient**
- **Level of awareness**
- **Skin colour**
- **De musset's Sign**
- **Jugular vein engorgement**
- **Breathing patterns**
- **Clubbing**
- **Posture**
- **Cyanosis**

On Examination

- **Postural Analysis**

- **Check for the posture of patient in different positions.**
- **Patient having COPD usually leans forward on hands/forearm and elevate the shoulder girdle.**

- **Chest Shape**

- **Barrel Chest**

- **Circumference of upper chest larger than lower chest.**
- **Sternum prominent**
- **AP Diameter greater than normal**
- **Seen in COPD**

On Examination

- **Chest Shape**

- **Funnel Chest**

- **Also called Pectus excavatum**
 - **Lower part of sternum depressed**
 - **Lower ribs flared out**
 - **Little upper chest movement in breathing**
 - **Patients are diaphragmatic breathers**

- **Pigeon Chest**

- **Also called Pectus Carinatum**
 - **Sternum prominent**
 - **Sternum protrude anterior**

On Examination

- **Chest Mobility**

- **Upper lobe expansion**

- **Place tip of thumb at mid sternal line at sternal notch**
 - **Extend your fingers above clavicle**
 - **Ask the patient to fully exhale and inhale deeply**

- **Middle lobe expansion**

- **Place the runner tip at Xiphoid process**
 - **Extend fingers laterally around ribs**
 - **Again ask the patient to breathe deeply**

On Examination

- **Percussion**

- **Technique use to assess lung density**
- **Place the middle finger of one hand against the chest wall along intercostal space.**
- **With the tip of the middle finger of opposite hand, firmly tap on the runner on chest wall.**
- **Repeat the same procedure all several points on right and left and anterior and posterior of chest wall.**

Sedative Physio

On Examination

- **Percussion**

- **The sound is dull/flat if greater than normal amount of solid matter is present.**
- **The sound is hyperresonant if there is greater amount of air than normal.**

Sedative Physio

On Examination

- **Ascultation**

- **Heart Sound and Lungs Sound should be assessed by ascultation.**
- **Normally two heart sounds are produced**
- **There can be various cause for the abnormal sound**
- **Abnormal 2nd sound of the heart can be either Split or single with reduced or loud intensity**
- **3rd sound in adults more than 30 years is abnormal**
- **4th sound is always abnormal**

Sedative Physio

On Examination

- **Areas for auscultation of Heart Sound**
 - **Mitral Area (Apex)**
 - 5th intercostal space just inside the midclavicular line
 - Murmur from mitral valve can best heard
 - **Tricuspid area**
 - Lower end of sternum near the ensiform cartilage
 - **Aortic area**
 - Also k/a Upper Right Parasternal Area or Second Rt. Intercostal Area
 - Murmur from aortic valve best heard

On Examination

- **Areas for auscultation of Heart Sound**
 - **Pulmonary area**
 - Also k/a Upper Lt. Parasternal or 2nd Lt. Intercostal area
 - ASD & Stenosis best heard
 - **Erb's area**
 - 4th Lt. Intercostal Area
 - Aortic Regurgitation , Pulmonary Regurgitation, HCM best heard.
 - Diastolic murmur is heard in both regurgitation
 - Systolic murmur is heard in HCM

On Examination

- **Breathe Sounds**

- **Areas of auscultation**

- **Apex of the lungs B/L**
 - **2nd intercostal space at midclavicular line**
 - **6th intercostal space**
 - **Laterally from axilla to 7th rib B/L**

Sedative Physio

On Examination

- **Normal Breathe Sounds**

- **Bronchial**

- High pitched
 - In both inspiratory and expiratory phases with a pause b/w them
 - Heard over trachea

- **Bronchovesicular**

- High pitched
 - In both inspiratory and expiratory phases without a pause b/w them
 - Location - Supraclavicular, Suprascapular, Parasternal, Interscapular

On Examination

- **Normal Breathe Sounds**

- **Vesicular**

- **Soft sound with no pause**
 - **Expiratory only one third**
 - **Location - Most of the chest area except trachea and between scapula**

Sedative Physio

On Examination

- **Abnormal Breathe Sounds**

- **Crackles**

- **Discontinuous low pitched sound**
 - **Mostly heard during inspiration**
 - **Heard as sound of rubbing hairs between fingers**

- **Wheeze**

- **High pitched and continuous sound**
 - **If it is present during inspiration - Bronchospasm**
 - **If it is present during expiration - Airway secretion**

On Examination

- **Abnormal Breathe Sounds**

- **Ronchi**

- **Low pitch and continuous sound**
 - **Can be heard during both phases**

- **Pleural rub**

- **Rubbing sound**
 - **can be heard during both phases**
 - **Due to rubbing of visceral and parietal pleura**

- **If bronchial sound is heard on peripheral lung field it is abnormal. It is called tubular breathe sound.**